



جوستاف روسي انترناشيونال - مصر (EPE) * طلب علاج أول مرة / متكرر

إسم المريض رباعياً : آمنه محمد حامد يوسف
المحافظة : اسوان الحى / المدينة : ادفو
رقم البطاقة : 25609232800525 رقم التليفون : 01158678588
العمر (تاريخ الميلاد) : 23-09-1956 المهنة : ربه منزل

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)

نتائج الأبحاث : (مرفق)

التشخيص النهائي : سرطان بالثدي (مبادرة)

خطة العلاج : علاج مبادرة سرطان الثدي العلاج المساعد لوقاية العظام للمرضى
الذين يتلقوا علاج هرموني حمض الزيلودرونك [14157]

التعليق على التشخيص : -----

خطاب إداري

هذا الرقم القومي يتبع منظومة التأمين الصحي الشامل جهة الاختصاص



Name: امنه محمد حامد يوسف
National ID: 25609232800525



تقرير طبي

السيد

تحية طبية وبعد,,

بشأن تقرير طبي للمريض

إيماء للطلب المقدم من

رقم طبي:

بقسم:

نتشرف بأن نوضح فيما يلي تقريراً عن حالته

تاريخ وساعة الخروج :
PM 01:17 05-07-2026

تاريخ وساعة الدخول :
-- --:-- yyyy-mm-dd

وبعد الكشف والفحوصات والمتابعة وجد أنه

المریضة تعاني من ورم غير حميد بالثدي و تحتاج الي Zoledronic acid for bone protection during hormonal therapy

الوقت :

-- --:--

التاريخ :

05-07-2026

الطبيب المعالج :
د/شيماء لاشين



الى من يشمه الامر / مستشفى جوستاف روسي

تحية طبية وبعد،،،،

بالإشارة الى قانون الهيئة العامة للتأمين الصحي الشامل رقم 2 لسنة 2018 ، والذي يتضمن بوجوب خضوع المقيمين بمحافظات التطبيق اجباريا بالمنظومة وفقا لمحل الاقامة ببطاقة الرقم القومي داخل احدي محافظات التطبيق، وبالإشارة الى بدء التشغيل الفعلي (2025\7) لمنظومة التأمين الصحي الشامل بمحافظة (اسوان)

برجاء التفضل بالعلم بأنه قد تم الاستعلام عن بيانات المواطن / امته محمد حامد يوسف تحمل بطاقة الرقم (25609232800525) المرفقه والتي قد تم اصدارها في تاريخ 2022\11 بمحل الاقامة في محافظة القاهرة بناء علي ما سبق في ان المستفيد المذكور من غير الخاضعين لمنظومة التأمين الصحي الشامل

وتفضلوا بقبول فائق التقدير والاحترام،،،

رئيس الادارة المركزية لشئون المستفيدين
د/احمد بركات
٥٥٨٣٨

تحديد في : 2026/03/30
ع.م. الادارة المركزية لشئون المستفيدين

فرع مصر الجديدة
لخدمة العملاء



جمهورية مصر العربية
بطاقة تحقيق المحاسبة

أمنه
محمد حامد يوسف
٢ حارة محمد إبراهيم
عين شمس - القاهرة

١٩٥٦/٠٠٣
٢٥٦٠٩٢٣٢٨٠٠٥٢٥

JQ8034374

٢٠٢٢/١١
٢٥٦٠٩٢٣٢٨٠٠٥٢٥
ربة منزل
أنش
مسند
البطاقة سارية حتى ٢٠٢٩/١١/٠٩

٢٠٢٩/١١/٠٩

04 / 05 / 2026

MDT FORM

Name:	Amna Mohamed Hammed Yousef	MRN:	3597
Age:	70 Y	ID:	
Menopausal Sta		Phone:	
NRS SCORE:		Marital Status:	
BMI:		Family History:	
CO Morbidity:		Reason Of MDT:	ADJUVANT AROMAZINE

Clinical Data:

BX:	
ER:	PR: HER2: KI67:

R

Sonomammography:

Mets Workup:

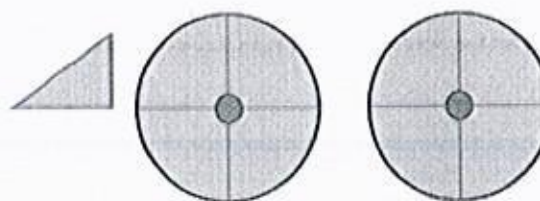
History:

Recommendations:
RECEIVED NACT AC+TAXOTERE WITH GOOD CLINICAL RESPONSE
underwent BCS+ALND
FOR ADJUVANT AI SHEE DEVELOPED SEVERE HYPERSENSITIVITY FROM AI
FOR AROMAZINE SINCE 2022

Recommendations:

MDT Outcome & Plan

ADJUVANT AROMAZINE AND ZOMETE EVERY 6 MONS



T N M

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psyc



Patient Name: Amna Mohamed Hamed Youssef	Date : 29/12/2022
Exam : ultrasound BREAST (BOTH SIDE)	Age : 66 Year(s)
Medical ID: 2724/2022	Pat. ID: 15504

Combined bilateral breast mammography and complementary ultrasound

Indication:

left breast pathologically proven malignant mass on NACT for assessment (patient had excisional biopsy outside our institution).

Technique

- Bilateral MLO and CC views
- Bilateral breast ultrasound

Report:

- Scattered fibro glandular density (ACR B)
- Left breast shows UOQ asymmetry with spiculations, associated skin retraction is appreciated
- Complementary ultrasound revealed left breast fibrotic band at the site of prior lumpectomy, seen at 2 o'clock 8cm from the nipple, measuring 0.7x3.5cm (was 2.7x3.7cm, in prior study dated 15/8/2022)
- No right breast solid or cystic masses
- Normal skin thickness, and nipple-areola complex of the right breast.
- Nonspecific right axillary LN
- No pathologically enlarged left axillary LNs
- Free left infra and supra-clavicular regions.

Opinion:

- Partial radiological response of the left breast UOQ malignant mass (BIRADS 6)
- Benign findings right breast (BIRADS 2)

Yours truly

Signature Dr. Engy Abdallah

29/12/2022 11:43 AM



Patient Name: Amna Mohamed Hamed Youssef	Date : 06/09/2022
Exam : TRIPHASIC ABDOMEN & PELVIS	Age : 66 Year(s)
Medical ID:	Pat. ID: 15504

For ultrasound correlatiuon POST CONTRAST MULTI-SLICE CT SCAN OF THE ABDOMEN & PELVIS WITH TRIPHASIC LIVER STUDY REVEALED: -

- The liver is average in size with few bilobar non enhancing focal lesions, the largest is seen at left hepatic lobe segment Iva measuring 13 x 12 mm all showing no enhancement throughout the triphasic study.
- Right hepatic lobe segment VIII calcified focal lesion mostly representing post granulomatous sequel.
- Patent homogenously enhancing portal vein and its main branches as well as its main tributaries.
- No intra-hepatic or extra-hepatic biliary radicles dilatation.
- Normal size & parenchymal texture of the spleen with no focal lesions seen.
- No pathological abdominal or pelvic lymph nodes.
- Bulky left adrenal gland.
- Normal size, parenchymal thickness & density of both kidneys with no stones or back-pressure changes.
- Normal appearance of the pancreas, aorta, I.V.C and right supra-renal.
- Smooth filling of the urinary bladder with no filling defects or diverticular outpouching.
- Normal appearance of the other pelvic organs.
- No abdominal or pelvic masses or collections.

The scanned skeleton:

- The scanned skeleton shows no definite worrisome lytic or sclerotic lesions.
- No vertebral structural collapse or extra-osseous soft tissue component.

OPINION:

- ***left breast malignant looking mass lesion.***
- ***Few bilobar hepatic focal lesions likely cysts for ultrasound correlation .***

Dr. Ahmed M. Rozeka, MSc

Dr. M. Gomaa, M.D.

Date : 06/09/2022	Patient Name: Amna Mohamed Hamed Youssef
Age : 66 Year(s)	Exam : ultrasound ABDOMEN& PELVIS
Pat. ID: 15504	Medical ID:

ABDOMINO-PELVIC ULTRASOUND EXAMINATION REVEALED:

Liver: -

- average in size (about 16 cm at the right MCL), smooth outline. multiple irregular area of fat deposition are noted for follow up. normal echopattern.
No intra hepatic biliary radicle dilatation.
- Portal vein is not dilated and patent .

Gall Bladder: -

- Average size , , normal wall thickness, no stones..
- CBD is not dilated.

Spleen: -

- Average size (About 10 cm), displaying homogenous echopattern.

Kidneys: -

- Are of normal size , site , well differentiated & with no gross stones or backpressure changes , no solid or cystic lesion.
- Para-aortic and pancreatic area: no gross lesion
- No ascites .

URINARY BLADDER: Normal filling. Normal wall thickness, no stones or diverticulae or solid lesion detected.

Uterus: is average size , normal endometrial line , no focal lesion.
No adnexal mass or cyst.

Opinion:

Multiple hepatic fat deposition area for follow up.

Signature: Dr. Doaa Abdelaziz
06/09/2022 11:16 AM



Patient Name: Amnah Mohamed Hamed Youssef	Date : 21/07/2022
Exam : C.T CHEST WITH CONTRAST	Age : 66 Year(s)
Medical ID:	

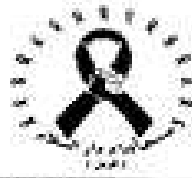
- The scanned skeleton shows no definite worrisome lytic or sclerotic lesions.
- No vertebral structural collapse or extra-osseous soft tissue component.

OPINION:

- *left breast malignant looking mass lesion.*
- *Few bilobar hepatic focal lesions likely cysts for US correlation .*
- *No definite pulmonary or osseous metastatic deposits.*

Dr. Mohammed Mohsen, MSc.

Dr. Amr Farouk, M.D.



Patient Name: Amnah Mohamed Hamed Youssef	Date : 21/07/2022
Exam : C.T CHEST WITH CONTRAST	Age : 66 Year(s)
Medical ID:	

POST CONTRAST MULTI-SLICE CT SCAN OF THE CHEST, ABDOMEN & PELVIS REVEALED: -

1. Chest:

- Left breast UOQ irregular speculated mass lesion measuring 26 cm seen reaching the focally thickened skin.
- No sizable pulmonary nodules.
- No pathological hilar or mediastinal lymph nodes.
- No evidence of pleural effusion, pleural thickening or masses seen.
- Average cardiac size. No pericardial effusion detected.
- Normal CT appearance of the thoracic aorta and the great vessels.

2. Abdomen and pelvis:

- The liver is average in size with few bilobar non enhancing focal lesions, the largest is seen at left hepatic lobe segment Iva measuring 26 X 12 mm.
- Right hepatic lobe segment VIII calcified focal lesion mostly representing post granulomatous sequel.
- Patent homogenously enhancing portal vein and its main branches as well as its main tributaries.
- No intra-hepatic or extra-hepatic biliary radicles dilatation.
- Normal size & parenchymal texture of the spleen with no focal lesions seen.
- No pathological abdominal or pelvic lymph nodes.
- Bulky left adrenal gland.
- Normal size, parenchymal thickness & density of both kidneys with no stones or back-pressure changes.
- Normal appearance of the pancreas, aorta, I.V.C and right supra-renal.
- Smooth filling of the urinary bladder with no filling defects or diverticular outpouching.
- Normal appearance of the other pelvic organs.
- No abdominal or pelvic masses or collections.

3. The scanned skeleton:



Patient Name: Amna Mohamed Hamed Youssef	Date : 15/08/2022
Exam : MAMMOGRAPHY BOTH	Age : 66 Year(s)
Medical ID: 2724/2022	Pat. ID: 15504

DIGITAL BILATERAL SOFT TISSUE MAMMOGRAPHY AND COMPLEMENTARY ULTRASOUND

Technique

- Bilateral MLO and CC views
- Bilateral 3D MLO DBT (Digital Breast tomosynthesis)
- Bilateral breast ultrasound

Report:

- ACR B
- Left Breast:
 - UOQ partially obscured high density lesion is seen , corresponding by U/S to 2 o'clock irregular hypoechoic solid mass , measuring about 3.7 x 2.7 cm with ductal extension towards the nipple for about 2.5 cm. No pectoral muscle involvement. It reaches 5 mm from skin surface and 7 cm distance from nipple.
 - Normal skin thickness and nipple-areola complex.
 - Solitary pathological level I axillary LN measures 1 cm.
 - No pathological enlarged infra or supraclavicular lymph nodes.
- Right Breast:
 - No suspicious micro calcifications or architectural distortion detected.
 - Complementary ultrasound no cystic or solid mass lesions.
 - Normal skin thickness, and nipple-areola complex with average caliber of the retro-areolar central ducts.
 - Nonspecific axillary LN.

Opinion:

- Left breast UOQ malignant mass with ipsilateral solitary pathological axillary LN (BIRADS 6) .
- Right breast : normal study (BIRADS 1)

Signature Dr. Peter Nashat

15/08/2022 5:53 PM



Name: Amna Mohammed Hamed

Date: 27/08/2022

Contrast enhanced Spectral Mammography

CLINICAL SUMMARY: LT pathologically proved IDCIII

CC & MLO low energy & recombined (post contrast) images

FINDINGS:

Background Uptake: mild background enhancement

ACR: C

LEFT BREAST:

- Low energy (Non contrast) images: UOQ irregular shaped heterogeneous dense mass lesion with spiculated margins with adjacent small lobulated intramammary lymph node located 1.3cm of the index lesion(confirmed by complementary US)
- Recombined (Post contrast) images: UOQ posterior one third heterogeneously moderately enhancing lesion measures 4.5x3.5x4.5cm located at about 10cm from the nipple.
- Complementary US: revealed level I pathological lymph node shows non uniform cortical thickness up to 6mm with effaced hilum, overall measurement 10x8mm.
- **RIGHT BREAST:**
- Low energy (Non contrast) images show no masses, asymmetry or distortion or suspicious microcalcifications
- Benign macrocalcifications are noted
- Recombined (Post contrast) images show no pathological uptake.
- Normal skin / nipple / areolar complex.

IMPRESSION :

LEFT BREAST: UOQ unifocal malignant mass lesion (BIRADS 6= pathologically proven)

RIGHT BREAST: benign (BIRADS 2= benign)

Best regards,

Dr. Marwa Ramzy, Msc



Patient Name: Amna Mohamed Hamed Youssef	Date : 28/11/2023
Exam : MAMMOGRAPHY BOTH	Age : 67 Year(s)
Medical ID: 2724/2022	Pat. ID: 15504

**BILATERAL BREAST MAMMOGRAPHY AND HIGH
RESOLUTION ULTRASOUND**

Indication:

Left breast conserving therapy

Report:

Left breast:

- Operative bed parenchymal distortion and scarring with no sizable mass
- No suspicious mass lesions, micro calcifications
- No sizable collection
- Diffuse edema pattern in the form of increased trabeculations and skin thickness.
- No regional lymphadenopathy

Right Breast:

- Scattered fibro glandular density (ACR B)
- No suspicious micro calcifications or architectural distortion detected.
- No spiculated mass
- Normal retro-areola ducts
- Normal skin thickness, and nipple-areola complex
- Nonspecific axillary LN.

Opinion:

- Post left BCS status showing no evidence of recurrence (BIRADS 2)
- Normal study of the right breast (BIRADS 1)

Best regards,

Dr. Amira H Radwan MD,
Breast Imaging consultant,

NCL Cairo University

28/11/2023 2:47 PM



Patient Name	: الأستاذة / امته محمد حامد يوسف	Request No.	: 13008980
Patient No.	: 25609232800525	Request Date	: 21/07/2022
Path No.	: SR-516/22	Verification Date	: 01/08/2022
Sex	: Female	Financial Category	: اورام دار السلام - نفقة الدولة
Age	: 65 Y	Department	: Out Patient
Referring Doc	:	Registered by	: marwa mohamed youss

Pathology Report

Clinical Data:

Left breast mass.

Nature of the specimen:

Slide revision.

Gross Examination:

2 blocks (664397) & 1 slide

Microscopic examination:

- Histological type: Invasive duct carcinoma.
- Grade (differentiation+ nuclear pleomorphism + mitosis): GIII (3+3+2)
- Angiolinvasion: No
- Ductal carcinoma in situ: No
- Additional pathologic findings: No

Diagnosis:

Left breast mass, Slide revision:

- Invasive duct carcinoma GIII.
- For IPT (ER, PR, Her-2, Ki67).

Prof. Dr. Lubna Omar

Dr. Abdulrahman Ibrahim

Verified by :

abdelrahman ibrahim

Patient Name :	الأستاذة / امته محمد حامد يوسف	Request No. :	13001671
Patient No. :	25609232800525	Request Date :	04/01/2023
Path No. :	44/2023	Verification Date :	24/01/2023
Sex :	Female	Financial Category :	اوارام دار السلام - نفقة التولية
Age :	66 Y	Department :	Out Patient
Referring Doc :		Registered by :	marwa mohamed youss

Pathology Report

Left breast cancer (post chemotherapy); Left BCS and ALNs specimens:

- Invasive duct carcinoma, grade III
- Free surgical margins
- Free axillary lymph nodes (0/13)
- Pathologic partial response
- Stage= ypT1 ypN0

Prof.Dr. Eman Gouda
Dr.Heba Mostafa

Verified by :
walaa ghoniem

Printing Date : 24/01/2023 11:28:28

Page 2 of 2

Printing By : [marwa mohamed youss](#)

Patient Name :	الأستاذ / امته محمد حامد يوسف	Request No. :	13001671
Patient No. :	25609232800525	Request Date :	04/01/2023
Path No. :	44/2023	Verification Date :	24/01/2023
Sex :	Female	Financial Category :	أورام دار السلام - نقطة التوبة
Age :	66 Y	Department :	Out Patient
Referring Doc :		Registered by :	marwa mohamed youss

Pathology Report

Clinical Data: Left breast cancer, post chemotherapy

Nature of the specimen: Left BCS and ALNs specimens, collected at 4/1/2023

Gross Examination

- Procedure: Left BCS specimen; measured 12x7x3 cm, covering skin measured 12x3 cm.
- Tumor extent
 - o Number: Ill defined rubbery gariysh white tumor bed site
 - o Size: 5x5x3 cm^{9,1}.
 - o Skin involvement: No
- Distance from margins: Least margin was 0.5 cm, (Deep margin)
- Lymph nodes number and size: Axillary fat measured 8x7 cm, entangling 13 nodules up to 1 cm.
- Additional pathologic findings: N/A
- Immunohistochemistry technique: N/A
- Frozen section: Free margins

Microscopic examination:

- Histological type: Invasive duct carcinoma, cellularity 50%
- Grade (differentiation+ nuclear pleomorphism + mitosis): III (3+3+2)
- Angioinvasion: Negative
- Ductal carcinoma in situ: Negative
- Lymph node involvement: Free axillary lymph nodes (0/13)
- Resection Margins: Free
- Histologic response to therapy: Pathologic partial response
- Additional pathologic findings: Microscopic largest invasive tumor diameter was 1 cm.

Diagnosis

Verified by :
walaah ghoniem

Printing Date : 24/01/2023 11:28:28

Page 1 of 2

Printing By : marwa mohamed youss

Dr.

Elia Anis Ishak

M. Sc. Ph. D. Pathology - Prof. of Pathology , Cairo University
Fellow, Armed Forces Institute of Pathology , USA
Member, United States Canadian Academy of Pathology

Dr.

Shady Elia Anis

M. Sc. MD Pathology , Ass. Prof. of Pathology , Cairo University
Fellow , Mass General Hospital , Harvard , USA
Member , United States Canadian Academy of Pathology

Patient Name : أمته محمد حامد يوسف

Analysis ID : 664397

Age : 65 Y : 9 M : 1 D

Gender : Female

Referred By Prof. Dr. : هدى عزت

Clinical Diagnosis : Left breast mass

Nature of Specimen : Biopsy

Receiving Date : 2022-07-07

Patient ID : 514337

PATHOLOGY REPORT

Gross :

Multiple irregular fibrofatty tissue pieces collectively measured 4x4 cm, with multiple irregular firm greyish white areas, measured up to 1.1x0.7 cm, totally submitted.

Microscopic :

Serial sections examined from the specimen received revealed pieces of tumour tissue formed of irregular groups of malignant ductal cells, showing marked nuclear pleomorphism and hyperchromasia. There is poor attempt at tubular formation.

There are moderate number of mitotic figures.

There is marked desmoplasia.

There is peritumoural lymphocytic infiltrate around 5%.

Lymphovascular invasion and Perineural invasion are absent in the sections examined.

There is focal tumoral necrosis.

No intraductal component presented.

Comment on surgical margins is not possible as the specimen was received fragmented.

Diagnosis :

Left breast mass, Biopsy

, INFILTRATING DUCT CARCINOMA, GRADE III.

Prof. Dr. Shady Elia Anis

Prof. Dr. Elia Anis Ishak



This report is electronically signed, any abrasion or modification would render it void.

1/1

عمارة رمسيس - بلوك أ - ميدان رمسيس - الدور السادس و السابع
تليفون : ٢٥٧٩.٩٣٥ - ٢٥٧٤١٧٤٢ - ٢٥٧٧٣٢٥٣



51

Patient Name	الأستاذة / امينة محمد حامد يوسف	Request No.	13008980
Patient No.	25609232800525	Request Date	21/07/2022
Path No.	SR-516/22	Verification Date	14/08/2022
Sex	Female	Financial Category	أورام دار السلام - نفقة الدولة
Age	65 Y	Department	Out Patient
Referring Doc.			

Registered by

.marwa mohamed yousse

IMMUNOHISTOCHEMISTRY REPORT

PR : Positive (6/8)
HER2 : Negative (score 0)
Ki67 : 23%

Prof. Dr. Lubna Omar
Dr. Abdulrahman Ibrahim

Verified by :

Dr. Marwa mahmoud

Printing Date : 14/08/2022 12:17:20

Page 2 of 2

Printed By : Ali mostafa



Patient Name : الأمستاة / امينة محمد حامد يوسف
Patient No. : 25609232800525
Path No. : SR-516/22
Sex : Female
Age : 65 Y
Referring Doc. :
Request No. : 13008980
Request Date : 21/07/2022
Verification Date : 14/08/2022
Financial Category : برنامج دار السلام - نفقة البولة
Department : Out Patient
IHC No. :
Registered by : marwa mohamed yousse

IMMUNOHISTOCHEMISTRY REPORT

Clinical Data:
Left breast mass.

Nature of the specimen:
Slide revision.

Gross Examination:
2 blocks (664397) & 1 slide
Sections prepared from paraffin block for immune- staining using Bench Mark ULTRA system for ER, PR, HER2, ER clone (1E2), PR clone (SP1), HER-2 clone (4B5), & Ki67 clone (30-9).

Microscopic examination.
• Histological type: Invasive duct carcinoma.
• Grade (differentiation+ nuclear pleomorphism + mitosis): GIII (3+3+2)
• Angioinvasion: No
• Ductal carcinoma in situ: No
• Additional pathologic findings: No
ER: Strong nuclear reaction in 70% of tumor cells (Allred score 8/8)
PR: Moderate nuclear reaction in 65% of tumor cells (Allred score 6/8)
Her-2: No membrane reaction of tumor cells (score 0)
Ki 67: 23 % nuclear reaction

Diagnosis:
Left breast mass, Slide revision:
-Invasive duct carcinoma GIII.
ER : Positive (8/8)

Verified by :
Dr.Marwa mahmoud

Printing Date : 14/08/2022 12:17:20

Page 1 of 2

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